

Future Cure for Cancer

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In this article, we evaluate adverse effects of medical treatments for cancer, discuss legal obstacles for finding cures, and propose future cure fighting method. We first show that the legal framework has created two conflicting fields of medicine.

A. Clinical medicine and cancer cause research

Medicine has developed two conflicting fields: traditional clinical medicine and cancer cause-discovery research.

The traditional clinical medicine, which concerns the treatment model. Main methods include surgeries (occasional use from 130–200 A.D./1700 to 1846, and increased use from 1846 to now), chemotherapy (from about 1946 to now) and radiotherapy (from about 1900 to now). Clinical medicine was developed under the control of the legal framework with dualism, reductionism, population approach, and common law models as key elements. As revealed by medical development histories, adoption and use of those traditional methods were under the influences of old and wrong cancer theories. While clinical medicine is concerned with the health of individual persons, its treatment methods are often developed by using population-based clinic trials.

Although epidemiological study method was used in ancient medicine, its use rapidly increases in cancer research after 1971. Most studies attempt to determine what factors are associated with diseases, and what factors may protect people against the disease. Since 1980, tens of thousands of studies have been published, which were intended to understand the roles of diet, pollutants, lifestyles, emotion, stress, exercises, lifestyles, etc.

Research activities intended to understand disease causes are not bound by the legal framework, and may be directed to anything. Researchers can try to find anything as predisposing factors, enabling/disabling factors, precipitation factors, and reinforcing factors. All those factors affect cancer growth speeds.

B. Wrong cancer treatments adversely affect the mankind

The legal framework of medicine has created great tension between the two fields. As a general rule, all cause-relating factors can affect the chances for a person to get a disease, but may not be used to cure the disease. We see product label statements like: X can make prostate healthy, followed by a disclaimer statement that X cannot cure disease. The law separates health from diseases as two distinctive states: a healthy prostate may have diseases, but a disease-free prostate may be unhealthy. A product label may say that correct lifestyle factors promote health and helps fight diseases, followed by a disclaimer statement, that they cannot cure diseases. This kind of conflicting statements can be found in health policies and laws, commercial circulars, and every cultural fiber.

The prohibition of use of cause-relating factors as cures has crippled medicine. If a cancer is caused by heavy metal, medicine does not stress the need to remove the metal, but cuts off tissues and organs; if a liver cancer is caused by a virus, medicine does not stress the need for eliminating the virus, but cuts the liver and poisons the body; if a digestive track cancer is caused by microbe's metabolic by-products, medicine does not stress how to change diet to rebuild a healthy gut flora, but cuts the stomach, intestines, the colon, etc, and poisons the organs and the brain; if a cancer is caused by a drug-induced autoimmune disease, medicine does not stress how to control systemic inflammation, but uses a surgical operation with an impact of making the inflammation worse; if a cancer is caused by a compromised immune system, medicine does not stress the need to boost the immune system, but uses a surgical operation with an impact to raise systemic inflammation, and uses chemotherapy to inhibit the immune system.

Medicine ignores the ancient teaching that uses right treatments for right disease causes. Medicine knows four critical points for controlling cancer growth: the immune system, systemic inflammation, emotional state, and tissue-ecosystem, but it continues using invasive surgeries to reduce reserve function and excess capacity of organs with an effect of worsening systemic inflammation, uses chemotherapy to inhibit or ruin the immune system, and uses radiation to ruin the Central Nerve System. In the last half a century, medicine has not done any research, which could enable doctors to focus on cause-related factors in treating cancer.

We have shown that all claimed benefits of all traditional medical treatments are based on chain comparisons among surgeries, chemotherapy and radiotherapy. The first reference, surgery, was established as a standard, and no one has ever done serious study to determine how surgery has affected patient lives, as compared with alternative non invasive options. Thus, all traditional treatments are selected like finding the tallest boy among a room full of little boys. The merit of surgery is based on two DECEPTIVE observations: removal of the tumor and frequently observed one-year plus survival. The absolute performance of surgery is UNKNOWN even though surgery has been widely used for more than a century.

Use of those traditional treatment methods has costed a huge number of lives. Medicine not only fails to find cures for cancer, but greatly shortens lives due to over- and excessive treatments. Numerous studies have found that the chance of cancer being overtreated is 25% to 60% [Welch and Black 2010; Jahn et al. 2015; Hoffman et al. 2014; Draisma et al. 2009]. Cancer treatments can cause a range of aging-like adverse effects, the hall-marks of which include genomic instability, telomere attrition, epigenetic alterations, mitochondrial dysfunction, loss of proteostasis, chronic low-grade inflammation and cellular senescence [López-Otín, 2013], and those cellular effects can be found in all vital organs. A study shows that lifespans of cancer survivals are cut shorter by estimated 30% [Cupit-Link et al. 2017] for certain cancer. The serious adverse effects of traditional treatments on cancer are established in both mechanisms, exemplar population data, and our kinetic studies.

In 2019, there is an estimated 1,762,450 new cancer cases diagnosed and 606,880 cancer deaths in the U.S. We estimate (based on $50/(100+50)=33\%$ rate) that, 200,000 annual deaths in the U.S. can be attributed to unnecessary treatments. Among the remaining 400,000 deaths, traditional treatments have shortened lives by various margins. Cancer patients of advanced stage cancers often have exhausted the reverse functions of vital organs due to the adverse impacts of cancer burden, and any additional adverse effects on such patients would trigger their deaths. We anticipate that invasive and toxic treatments cause fragile patients to die sooner during treatment. The number of cancer survivors in the U.S. is between 15 to 20 millions now. Those people will lose lifespans by large margins.

In China, there are 4.51 million cases and 3.04 million deaths for 2020. We estimate that traditional treatments may cause about 1 million annual deaths. In the world, about 9 billion people are dying from cancer. A large number of deaths are caused by traditional treatments while the remaining survivors lose considerable parts of lifespans.

None of the federal agencies can address those issues.

C. Remote hope for finding cancer cure due to the huge legal obstacles

Fighting cure is not a matter only for cancer patients. Cancer is part of somatic cell evolution, and part of human aging process. Tumor development must be part of normal human life in every person. Cell mutations may accumulated at the birth and steadily added in the entire life. By the age of 70, tiny undetectable tumors most probably exist in the body; and by the age of 85, tumors could be found in the body at nearly certainty. Only differences between "normal people" and cancer patients are cancer growth speeds. Medicine will not succeed by cutting off tumors, poisoning and burning the whole body.

The failure of finding cures has forced cancer research to shift its focus to lifestyle factors. This is reflected in a September 3, 2019 article by NCI Staff, "Altering Diet Enhances Response to Cancer Treatments in Mice" This move shows that NCI used lifestyle factors as part of cure for cancer. The concept of using specific dietary changes to enhance cancer treatment "is really at the very early stages," said Jason Locasale, Ph.D., who led the study. This change reflects that NCI is willing to consider treatment options outside the traditionally used options.

This change also implies that future medicine would be based on individual persons, but not the population [Ogino et al. 2012 and 2013]. All cancer patients have different lifestyles, it is impossible to use the same lifestyle to all cancer patients. This change also implies that someday doctors would start writing up lifestyle prescriptions as part of cancer treatments.

This strategical change is welcome, but it took nearly half a century to take the first step. The slow change means that several generations of people in our time will miss opportunities to see cures for cancer.

Using lifestyle prescription as cancer cure will met with huge obstacles. NCI can use federal funds to support research, but this kind of research will not attract private research organizations. If a cure is found, no body can make money, and thus no body will promote it. In addition, most cancer researchers may avoid doing this kind of research because they cannot use the double blind ramanmized clinic trial designs. Without using double-blind ramanmized experimental protocol, their research articles on treatment methods cannot be published. If they have developed a cancer fighting program, they cannot patent it. No body will do business that loses money. Even if lifestyle prescriptions were developed, they will not attract clinical doctors. No body can make money from this treatment method. The treatment model is unworable under the current medical practice model because doctors cannot write up a lifestyle prescription in a 15 minutes visit. It takes a few days to figure out a patient's lifestyle details.

For those who are at age of 40 to 60, preventing cancer and fighting cancer is their lifetime matter and part of their lives. In the next 20 to 60 years, effective cancer prevention and treatment methods are important to them. Unless the law is changed, cures for cancer will not come from medical research because it took half a century to take the first step and perhaps another half century to take another step. Therefore, people in those ages must find new ways to prevent and cure cancer. A common wish of people dying from incurable diseases is that medicine had a cure. If we want to put an end to the repeating wishing, everyone must do his part to change the legal framework. The law will not change on its own.

D. Self-help cancer treatment programs

Solving cancer problems depends on our own efforts and starts with rejecting the idea of instant removal of the tumor. Removing a tumor will cause more tumors to grow at much faster speeds. The current medical strategy can be used to deal with life emergencies, but should not be used to control cancer burden. For patients with physical ability to fight, we recommend the following strategy.

1. The real problem in cancer patients is that the tumor grows too slow. The right strategy is to develop a comprehensive cancer fighting program to slow down its growth speed.

2. Find the factors that can slow down the growth of cancer, by relying upon the tens of thousands of medical studies published since 1980. Pay attention to body temperature, exercise, toxins, emotion/stress, diet, emotion, stress, etc. that affect cancer growth speeds. Identifying all factors that most probably affect the patient's specific cancer. This can be done by examining the patient's lifestyle and habits in light of all known risk factors or the HYPOTHETICAL LIFESTYLE formed in evolution. Cause factors for different patients suffering the same type of cancer are often different.

3. Develop a comprehensive cancer-fighting plan or program: measures to be included in the plan include regulating mental and emotional state, changing diet or using special diet, doing exercise or special exercise, changing lifestyles, getting rid of bad thinking habits, getting rid of bad working habits, changing living environment, etc. The measures, which may involve tens to hundreds of factors, should include cause factors that are most relevant to the patient. Exercises must be included because it has a wide, universal, and powerful effects on inhibiting cancer growth. However, different exercises have different effects on different types of cancers.

4. Cancer cells can complete a cell division cycle in 24 hours. The measures of the cancer-fighting plan is intended to create a "tissue ecosystem" that is hostile to cancer cell division on each day. The plan is designed to deliver sufficient "instantaneous" effects, "time-averaged" daily effects, and multi-stage effects. Exercise plus dietary and emotional measures should deliver sufficient force to cause cancer cells to stop division and create a tissue ecosystem that will not allow dividing cancer cells to complete cell division.

5. The plan must be executed diligently on a daily basis. Two days of hard work followed by three days of breaks will achieve little, which is the root cause of failure. It takes a long time to cure cancer, don't stop too early.

6. Periodically evaluate the effectiveness of the plan. If the cancer continues growing, raises the daily "instantaneous" effects, the daily "time averaged" effects and multi-stage effects, consider optimizing exercises by using better varieties and performance times, and use better combinations of better measures. In optimizing exercise, Qigong, Tai-chi, Yoga, etc. may be used to deliver stronger daily effects.

This method uses special exercise plus multi-factor methods in place of classical medical treatment methods, and uses the traditional treatment methods as auxiliary methods.

Avoid anything, activities, events, etc. that can trigger strong inflammation reaction or lasting adverse emotional reaction.

At present, no cancer expert has proposed to treat cancer by using only lifestyle factors, because they correctly believe that the effect of taking any single measure is too slow. Based on the sensitivity of cancer cell growth rate to various factors, we have used kinetic simulation calculations to prove that the combination of multiple factors can achieve the effect of suppressing the growth of cancer cells of most genetic types. We also predict if the measure specifically addresses the cause factor of a special cancer patient, correcting such a factor alone is enough to stop cancer growth.

Right-type, optimized and well executed exercise can effectively suppress the growth of cancer cells. This can be estimated from the fact that exercise can significantly reduce cancer mortality and recurrence rates. Cancer can heal itself, be inhibited by unknown factors, or be slowed down by using multiple lifestyle factors. There are hundreds of well-documented success cases, and millions of undocumented stories.

Difficulty lies in the second step above. Although luck is a factor, cancer outcomes depend on individual wisdom, persistence, and hard work.

Do not try to assess the effectiveness of the plan by using your own feelings. It is very common that a patient will experience mild adverse reactions. A day when a person feels good, may be the worst day of adding millions of new cancer cells; but the day when the person feels uncomfortable may be the day of eliminating millions of cancer cells. The state of human perception has nothing to do with the rise or decline of cancer cells. People who do not understand cancer cell changing dynamics and factors' action mechanisms should not use time correlation and subjective perception to judge the effectiveness of each measure.

Do not use democratic methods to assess the merit of any measure. If ordinary people can understand the nature of the disease and effects of each treatment measures, medicine could have found cure. Support of online friends may inspire the patient and help the fight against cancer. However, we must point out that no one would provide any suggestion that contradicts the law and medical culture. Because the probability of bad cancer outcomes is very high, friends, relatives, and even spouses are not in a position to give honest advises. "Listening to the doctor" is the safest advice. We have seen that many cancer patients have relied on democratic verdicts to shorten their lives, and they even do not know.

Do not listen to opinions of the American Cancer Society and federal agencies that have a legal obligation to protect the classic clinical treatments,

preclude any treatments that are compatible with evolution, and lead patients to the wrong paths. They are often used “pseudoscience” to confuse patients. When we expose medical problems or the bottom line, they can only be silent because they can do nothing about the law.

The average person’s program can only achieve 10%-50% the potentially maximum performance. Even though, it could effectively stop cancer growth or reverse the cancer in most cases.

Finally, we have to say by law: "Only surgery, chemotherapy, radiotherapy and things approved by the FDA can cure cancer; all evolution-compatible alternative treatments such as emotional regulation, dietary changes, detoxification, exercise, dancing, Qigong, Taichi, meditation, laughter, etc., cannot cure cancer." This is a big joke in the world in the scientific era of 2019.

For any question, please contact John Wu at 202-560-3000, tempaddr2@atozpatent.com (the preferred contact).

Note: Those findings are suppressed by medical journals while general media rely on articles of medical journals. Most federal agencies keep silence to those issues. Some responses indicate the NIH has no authority to investigate or stop any model mistake in medicine. Thus, no body can do anything to stop such a serious mistake that will continue hurting human civilization.

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